

Fall Prevention Policy

Internal policy / SOP - for demo purposes only

1. Purpose

This policy establishes the procedures for identifying residents at risk for falls, implementing preventive interventions, and responding to fall incidents within the facility. It applies to all clinical and direct-care staff.

2. Fall Risk Assessment

Every resident must be assessed for fall risk on admission, after any change in condition, after any fall, and at minimum quarterly. The Morse Fall Scale shall be used as the primary screening instrument. Residents scoring 45 or higher are classified as high risk; scores of 25 to 44 are classified as moderate risk; scores below 25 are low risk. Risk classification must be documented in the resident's chart and reflected in the care plan.

3. Fall-Risk Alerts

Residents classified as high or moderate fall risk shall have a fall-alert indicator placed in their electronic record and a visible identifier (yellow wristband and door magnet) to inform all staff. Staff entering the room must observe gait, positioning, and any visible hazards. The alert remains active until the next quarterly reassessment downgrades the risk.

4. Preventive Interventions

For high-risk residents the following interventions are required: bed in lowest position, non-slip footwear at all times when out of bed, call light within reach, scheduled toileting every two hours, gait-belt assistance for all transfers, and night-light in bathroom. Moderate-risk residents receive scheduled toileting and the lowered-bed measure. Physical therapy consultation is recommended for any resident with two or more documented falls in a 90-day window.

5. Response to a Fall

Any staff witnessing or discovering a fall must (a) ensure the resident is not in immediate danger; (b) perform a focused neurological and musculoskeletal assessment before moving the resident; (c) notify the charge nurse immediately; (d) document the fall in the incident-report system within one hour, including location, severity, suspected cause, witnesses, and interventions; (e) notify the resident's physician and family within four hours.

6. Severity Classification

Falls are classified as minor (no injury or superficial only), moderate (skin tear, bruise, abrasion, or limited functional impact), or severe (fracture, head injury, loss of consciousness, or any injury requiring transfer to acute care). Severe falls require notification to the facility medical director within 24 hours and a root-cause review within 7 days.

7. Post-Fall Follow-Up

All falls require a care-plan review within 72 hours. The care plan must document any change in fall-risk classification, additional interventions added, and a clear timeline for reassessment. A resident with two or more falls in 90 days is automatically referred for physical therapy evaluation and is reviewed at the next interdisciplinary team meeting.

8. Documentation

All assessments, interventions, incidents, and follow-up actions shall be documented in the resident's medical

record. Documentation must be completed by the end of the shift on which the action occurred. Failure to document is treated as a quality-of-care deficiency under this facility's compliance policy.